

(W)200mm

(L)250mm

pharmaniaga™

Tetracycline

Capsule

DESCRIPTION

Yellow / black, size '2', hard gelatin capsule with Pharmaniaga icon and '250' markings. Capsule shell is of bovine source.

COMPOSITION

Each capsule contains Tetracycline hydrochloride 250 mg.

ACTIONS

Tetracyclines are broad-spectrum bacteriostatic agents and act by inhibiting protein synthesis by blocking the binding of aminoacyl tRNA (transfer RNA) to the mRNA (messenger RNA) ribosome complex. Reversible binding occurs primarily at the 30 S ribosomal subunit of susceptible organisms. Bacteria cell wall synthesis is not inhibited.

Tetracycline is 75-77% absorbed orally and is readily distributed to most body fluids, including bile, sinus secretions, and synovial, pleural, ascetic, and gingival crevicular fluids. Tetracycline tends to localise in bile, liver, spleen, tumours and teeth; it also crosses the placenta and is distributed into breast milk.

Tetracycline has a normal half-life of 6 to 11 hours. Two to three days may be necessary to achieve therapeutic concentrations of tetracycline.

Tetracycline is bound in varying degrees to plasma proteins. It is concentrated by the liver on the bile and excreted in the urine and faeces at high concentrations and in a biologically active form. Tetracycline is slowly removed by haemodialysis.

INDICATIONS

Tetracycline is indicated for infections caused by chlamydia (psittacosis, salpingitis, urethritis and lymphogranuloma venereum), rickettsia (including Q-fever), mycoplasma (respiratory and genital infections), and spirochaete, *Borrelia burgdorferi*, (Lyme disease). They are also used in acne, in destructive (refractory) periodontal disease, in exacerbations of chronic bronchitis (due to activity against *H. influenzae*), and for leptospirosis in penicillin hypersensitivity.

CONTRAINDICATIONS

This drug is contraindicated in patients who have shown hypersensitivity to any of the tetracyclines and in pregnant women.

ADVERSE REACTIONS

Gastrointestinal : Pseudomembranous colitis, anorexia, nausea, vomiting, diarrhoea, glossitis, dysphagia, enterocolitis, pancreatitis, and inflammatory lesions (with monilial overgrowth) in the anogenital region, increases in liver enzymes, and hepatic toxicity have been reported rarely. Rare instances of oesophagitis and oesophageal ulceration have been reported in patients taking the tetracycline-class antibiotics in capsule and tablet form. Most of these patients took the medication immediately before going to bed.

Skin : Maculopapular and erythematous rashes. Exfoliative dermatitis has been reported but is uncommon. Fixed drug eruptions, including balanitis, have been rarely reported. Photosensitivity is discussed in warnings.

Renal toxicity : Rise in BUN has been reported and is apparently dose related.

Hypersensitivity reactions : Urticaria, angioneurotic oedema, anaphylaxis, anaphylactoid purpura, pericarditis and exacerbation of systemic lupus erythematosus.

Blood : Haemolytic anaemia, thrombocytopenia, neutropenia and eosinophilia have been reported.

CNS : Pseudotumor cerebri (benign intracranial hypertension) in adults and bulging fontanels in infants. Dizziness, tinnitus, and visual disturbances have been reported. Myasthenic syndrome has been reported rarely.

Other : When given over prolonged periods, tetracyclines have been reported to produce brown-black microscopic discolouration of thyroid glands. No abnormalities of thyroid function studies are known to occur.

WARNINGS

THE USE OF DRUGS OF THE TETRACYCLINE CLASS DURING TOOTH DEVELOPMENT (LAST HALF OF PREGNANCY, INFANCY AND CHILDHOOD TO THE AGE OF 8 YEARS) MAY CAUSE PERMANENT DISCOLORATION OF THE TEETH (YELLOW-GRAY-BROWN).

This adverse reaction is more common during long-term use of the drugs but has been observed following repeated short-term courses.

Enamel hypoplasia has also been reported.

TETRACYCLINE DRUGS, THEREFORE, SHOULD NOT BE USED IN THIS AGE GROUP UNLESS OTHER DRUGS ARE NOT LIKELY TO BE EFFECTIVE OR ARE CONTRAINDICATED.

If renal impairment exists, event usual oral or parenteral doses may lead to excessive systemic accumulation of the drug and possible liver toxicity. Under such conditions, lower than usual total doses are indicated and, if therapy is prolonged, serum level determinations of the drug may be advisable.

Photosensitivity manifested by an exaggerated sunburn reaction has been observed in some individuals taking tetracyclines. Patients apt to be exposed to direct sunlight or ultraviolet light should be advised that this reaction can occur with tetracycline drugs, and treatment should be discontinued at the first evidence of skin erythema.

The anti-anabolic action of the tetracyclines may cause an increase in BUN. While this is not a problem in those with normal renal function, in patients with significantly impaired function, higher serum levels of tetracycline may lead to azotemia, hyperphosphatemia, and acidosis.

Pregnancy

Results of animal studies indicate that tetracyclines cross the placenta, and are found in foetal tissues and can have toxic effects on the developing foetus (often related to retardation of skeletal development). Evidence of embryotoxicity has also been noted in animals treated early in pregnancy. Use is not recommended in the last half of pregnancy.

PRECAUTIONS

Breast-feeding

Tetracyclines are distributed into breast milk; although tetracycline may form non-absorbable complexes with breast-milk calcium use is not recommended because of the possibility of their causing permanent discoloration of teeth, enamel hypoplasia, inhibition of linear skeletal growth, photosensitivity reactions, and oral and vagina trush in infants.

Geriatrics

No information is available on the relationship of age to the dosage of tetracyclines in geriatric patients.

DRUG INTERACTIONS

Anticoagulants : Concurrent use may cause increased effects of oral anticoagulants.

Antacids, calcium supplements, choline and magnesium salicylate, iron supplements, magnesium-containing laxatives, sodium bicarbonate: Concurrent use may result in formation of non-absorbable complexes, also, concurrent use with antacids or sodium bicarbonate may result in decreased absorption of oral tetracyclines because of increased intragastric pH; patients

should be advised not to take these medications within 1 to 3 hours of oral tetracyclines.

Cholestyramine, colestipol : Concurrent use with cholestyramine or colestipol may result in binding of oral tetracyclines, thus impairing their absorption; an interval of several hours between administration of cholestyramine or colestipol and oral tetracyclines is recommended.

Oral oestrogen-containing contraceptive : Concurrent long-term use with tetracyclines may result in reduced contraceptive reliability and increased incidence of breakthrough bleeding.

DOSAGE AND ADMINISTRATION

Tetracycline should be taken on an empty stomach 1 hour before or 2 hours after meals with 1 full glass of water.

Anti-acne : 500 mg to 2 g daily in divided doses initially in moderate to severe cases as adjunctive therapy. When improvement is noted (usually after 3 weeks), dosage should be reduced gradually to a maintenance dose of 125 mg to 1 g daily. Adequate remission of lesions may also be possible with alternative-day or intermittent therapy.

Brucellosis : 500 mg every six hours for 3 weeks, given concurrently with 1 g of streptomycin intramuscularly every twelve hours the first week and once a day the second week.

Gonorrhoea : 500 mg every six hours for 5 days.

Syphilis: 500 mg every six hours for 15 days (early syphilis) or for 30 days (late syphilis).

Uncomplicated urethral, endocervical, or rectal infection caused by Chlamydia trachomatis : 500 mg four times a day for at least 7 days.

Usual adult prescribing limits : Up to 4 grams daily.

Infants and children to 8 years : Use not recommended.

OVERDOSAGE

No specific antidote. Gastric lavage plus oral administration of milk or antacids.

ROUTE OF ADMINISTRATION

Oral.

STORAGE CONDITIONS

Store below 30°C.
Protect from light.
Keep container tightly closed.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In bottle of 100 capsules (for KKM tender only).
In blisters of 100 capsules.

Date of revision: 27th April 2017

PRODUCT REGISTRATION HOLDER/ MANUFACTURER: PHARMANIAGA MANUFACTURING BERHAD (60016-D)

11A, Jalan P/1, Kawasan Perusahaan Bangi, 43650 Bandar Baru Bangi, Selangor Darul Ehsan, Malaysia.

PRP 0220.7 270417

attn	customer	Pharmaniaga Manufacturing Berhad	date	05.05.17	ARTWORK LOG																					
1 colours	process colour:	Black	Please Chop & Sign For Approval		<table><thead><tr><th>Version no.</th><th>Date</th><th>Reason for Change</th></tr></thead><tbody><tr><td>01</td><td>05.05.17</td><td>- Add in Route of Administration and amend PRC no.</td></tr><tr><td></td><td></td><td></td></tr><tr><td></td><td></td><td></td></tr><tr><td></td><td></td><td></td></tr><tr><td></td><td></td><td></td></tr><tr><td></td><td></td><td></td></tr></tbody></table>	Version no.	Date	Reason for Change	01	05.05.17	- Add in Route of Administration and amend PRC no.															
Version no.	Date	Reason for Change																								
01	05.05.17	- Add in Route of Administration and amend PRC no.																								
size	(L)250 x (W)200mm																									
description	Tetracycline Leaflet																									
 FocusPrint SDN BHD		t: 603-8766 6030 f: 603-8766 6033 (Factory)																								
		website: www.focusprint.info																								
artwork prepared by: cynthia yap		email: graphic@focusprint.info (graphic Dept)		date:																						
IMPORTANT ! Please note that this colour print is for visual purpose only. Output colour might differ in actual production.																										